Three Mistakes Families Make When a Care Decision Is Close and Not Everyone Agrees

How each one shows up, where it comes from, and the habit to put in its place.

These are the three habits the Family Care Roadmap watches for at this step. None of them is a failing, and none means anyone cares less. Each has a plainer habit to put in its place.

1 Assuming everyone wants the same outcome

WHAT IT LOOKS LIKE Everyone says “we just want what's best,” and the conversation moves on. A few days later it turns out that “best” meant one thing to your brother, another to your sister and a third to you.

WHY IT HAPPENS “What's best” sounds like agreement, so nobody checks. The Family Caregiver Alliance, writing for brothers and sisters caring for a parent, notes that siblings can see a parent's situation differently, and that these differences are common. [1]

> **WHAT TO DO INSTEAD** Ask the question out loud: where does the family agree, and where doesn't it? Write both lists down in words each person would accept. You don't have to close the gap this week. The Family Caregiver Alliance says not every issue in caregiving and decision-making can be solved, and that a family meeting is not a one-time event. [2]

2 Letting the loudest voice define the plan

WHAT IT LOOKS LIKE One relative has done the reading, holds a firm view and makes the case well. The others go quiet, some because they agree and some because they're tired. By the end of the call it is being called the plan.

WHY IT HAPPENS A confident voice is a relief when everyone is worn out and a decision is close. Quiet then gets counted as agreement.

> **WHAT TO DO INSTEAD** Before the options come up, ask which questions must be answered before the family decides, and give every person the same questions and the same room to answer. The Family Caregiver Alliance says a successful family meeting gives everyone a chance to be heard, and that a neutral outside facilitator often helps when a family is divided on a big issue. [2]

3 Losing your loved one's preferences in a rushed decision

WHAT IT LOOKS LIKE The talk is all about your loved one and none of it is with them. “She'd never want that” and “she'll be fine with it” are said across the table, and nobody can remember when your loved one last said either.

WHY IT HAPPENS There's a date coming, and it can feel kinder or quicker not to put the question to them. AARP's planning guide for families observes that in a crisis, big decisions may be driven by assumptions. [3]

> **WHAT TO DO INSTEAD** Ask the first question first: what does your loved one want? The National Institute on Aging says the best way to know what someone needs is to ask them directly. Write the answer in their own words, put it at the top of the page, and read it aloud before anyone else speaks. [4]

The habit under all three

Write down what was actually said and who said it, starting with your loved one. Compare options after that page exists.

What this guide doesn't do

It doesn't say who in your family is right, what you should decide, or what your loved one needs. It names three habits, and a plainer one to put in place of each.

Ready for the next step?

You know the three habits to watch for. The care interview gives the family something firmer than anyone's opinion to work from: a structured look at your loved one's situation, area by area, turned into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

The observations about relatives seeing a situation differently, giving everyone a chance to be heard and decisions driven by assumptions come from the sources below, as does the advice to ask your loved one directly. The three mistakes themselves are the Family Care Roadmap's.

1. Family Caregiver Alliance, Caregiving with Your Siblings
2. Family Caregiver Alliance, Holding a Family Meeting
3. AARP, Prepare to Care: A Planning Guide for Families
4. National Institute on Aging, Does an Older Adult in Your Life Need Help?

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